



ANXIETY DISORDERS

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Normal Anxiety



Anxiety is a normal human being feeling. It helps many adaptive functions & can improve the functioning of the individual

Anxiety is a diffuse , unpleasant, vague sense of apprehension often accompanied by autonomic symptoms

In addition to visceral & motor effects ; anxiety affect thinking ,perception and learning

When anxiety increases to a degree that cause suffering or affects the functioning of the individual it becomes pathological anxiety

Introduction

Anxiety disorders are a group of mental disorders characterized by the presence of anxiety as the main and prominent symptom

They make up one of the most common groups of psychiatric disorders

Epidemiological studies found that 1 in 4 people has met the diagnostic criteria of at least one of the anxiety disorders and that 12-months prevalence rate is 17.7%

Women are more affected than men (30.5% for women & 19.2% for men)

ANXIETY DISORDERS STATISTICS

Anxiety disorders affect
40 million
adults in the U.S.



For many people, symptoms begin to develop
before they are **21** years old.

Generalized anxiety disorders is the
most common
cause of workplace disability.



Theories of pathological anxiety

1. Psychological theories:

Three major schools of psychological theories have contributions about the causes of anxiety:

A- Psycho-analytic theory: anxiety is the result of conflict between unconscious sexual or aggressive wishes and corresponding threats from the superego or external reality. In response to this signal, the ego mobilizes defense mechanisms to prevent unacceptable thoughts and feelings from emerging into conscious awareness.

B- Behavioral theories: Anxiety is a conditioned response to specific environmental stimuli. It also result from imitation of anxiety responses of other people (social learning theory)

C- Existential theories: these provide models for generalized anxiety disorder. The central concept of this theory is that people becomes aware of feelings of profound nothingness in their lives. Anxiety is their response to the vast void in existence & meaning.

2. Biological theories:

A- Autonomic nervous system: stimulation of autonomic n.s. causes certain symptoms ;like cardio-vascular symptoms, muscular, GIT, &respiratory

B- Neurotransmitters: the three major NT associated with anxiety on the basis of animal studies & response to drugs are: norepinephrine , serotonin(5HT), and Gamma-amino-butyric acid (GABA)

C- Brain imaging studies:

Structural: CT & MRI have found many abnormalities in the brains of patients having anxiety disorders, for example: increase in the size of cerebral ventricles, specific deficits in the right temporal lobe, abnormal function of the right cerebral hemisphere but not the left which means cerebral asymmetry

Functional: PET, SPECT & EEG of patients with anxiety disorders have reported abnormalities in the frontal cortex, the occipital & temporal areas and other regions

Neuro-anatomical considerations: The raphe nucleus and the locus coeruleus which are the main areas important in anxiety projects with areas in the limbic system and cerebral cortex

D- Genetic studies:

Has produced solid data that at least some genetic component contributes to the development of anxiety disorders

Classification of anxiety disorders

1. Generalized Anxiety Disorder
2. Specific phobia
3. Social phobia
4. Panic disorder
5. Agoraphobia
6. Anxiety due to general medical condition
7. Substance –induced A.D
8. A.D not otherwise specified (like mixed anxiety-depressive disorder)
9. Separation Anxiety Disorder
10. Selective Mutism

Generalized anxiety disorder

Is abnormal fear that is out of proportion to any external stimulus

GAD is an excessive anxiety & worry about several events or activities for majority of days during at least a 6-month period

Is characterized by intense pervasive worry over virtually every aspect of life (job performance, health, marital relations, and social life)

The worry is difficult to control and is associated with somatic symptoms such as muscle tension , irritability, difficulty sleeping and restlessness

Symptoms of GAD

A- Psychological symptoms:

Fearful anticipation, irritability, sensitivity to noise, restlessness, poor concentration and worrying thoughts

B- Physical symptoms:

GI symptoms

Respiratory

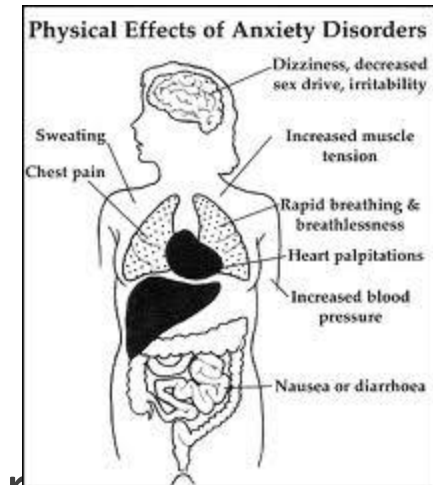
CVS

Genito-urinary

Neuro-muscular

C- Sleep disturbance: insomnia, night mares

D- Other symptoms: depression, obsessions and depersonalization



Epidemiology

GAD is a common condition

1-year prevalence range from 3-8%

It is commonly coexist with other mental disorders like social phobia ,specific phobia, panic disorder & depressive disorder

Women to men is 2:1

The age of onset is difficult to specify but is mostly at the 20s

Course & prognosis

As many as 25% eventually GAD is a chronic condition that may be life-long

As many as 25% eventually experience panic disorder

An additional high percentage are likely to have major depressive disorder

Differential diagnosis

A- Medical disorders:

1. **Cardio-vascular diseases:** anemia, angina, congestive heart failure, mitral valve prolapse, MI, paradoxical atrial tachycardia
2. **Pulmonary disease:** asthma, hyperventilation, and pulmonary embolism
3. **Neurological diseases:** CVA, epilepsy, Huntington's disease, infections, Menier's disease, migraine, tumors, MS,...
4. **Endocrine diseases:** Addison's disease, carcinoid syndrome, Cushing disease, DM, hyperthyroidism, hypoglycemia, hypoparathyroidism, pheochromocytoma, premenstrual syndrome.

5. **Drug intoxication:** amphetamine, anticholinergics, cocaine, hallucinogens, marijuana, theophylline, nicotine

6. **Drug withdrawal:** alcohol, opiates, sedatives
7. **Other conditions:** anaphylaxis, B12 deficiency, electrolyte disturbances, heavy metal poisoning, uremia, systemic infections

B- Mental disorders:

- 1. Malingering**
- 2. Factitious disorder**
- 3. Hypochondriasis**
- 4. Depersonalization disorder**
- 5. Social & specific phobia**
- 6. Posttraumatic stress disorder (PTSD)**
- 7. Depressive disorder**
- 8. Schizophrenia**

Treatment

The most effective treatment of GAD is probably one that combines psychotherapy , pharmacotherapy and supportive therapy

Psychotherapy:

1. Cognitive-behavioral therapy
2. Insight oriented therapy
3. Supportive therapy

Pharmacotherapy:

1. Benzodiazepines
2. Serotonergic agents: buspirone
3. Other drugs: tricyclic antidepressants, b-adrenergic receptor antagonists like propranolol



PHOBIC ANXIETY DISORDERS

Neurotic states with an abnormally intense dread (pathologically strong fear) of a particular events or things which would not normally have that effect

Types: Simple phobia, Agoraphobia, Social phobia



Simple phobia



Some specific object or situation causes the person unreasonable anxiety

It's more common among women than men. Typical onset is in childhood with most cases occurring before age 12

Specify Types of Phobias;

- a- Animal (snake, dog, spider...)
- b- Natural Environmental (heights, storms, water)
- c- Blood-injection injury
- d- Situational (airplanes, elevators, enclosed places)
- e- Other (situations that may lead to choking, vomiting)

Agoraphobia

A morbid fear of public places and/or of open spaces, but often used for a fear of:

Shops & supermarkets

Buses & trains

Crowds

Any place that can not be left suddenly (middle of theater)

Onset usually between ages 15-35, more common among women

Social phobia



A fear of situations in which the individual may be observed by other people e.g.: (Restaurants, dinner parties, speaking in class)

Also a fear that the individual may behave in a manner that will be humiliating or embarrassing. The disorder may be generalized or limited to specific situations

Onset usually in adolescence before age 25, equally common in men & women



Treatment of Specific Phobia

A. The primary treatment is behavioral therapy:

A commonly used technique is systemic desensitization, consisting of gradually increasing exposure to the feared situation, combined with a relaxation technique such as deep breathing.

B. Beta-blockers may also be useful prior to confronting the specific feared situation.

Treatment of Social Phobia

- A. SSRIs, such as paroxetine 20-40 mg/day or sertraline 50-100 mg/day, are first-line medications for social phobia. Venlafaxine 75-225mg/day may also be used. Benzodiazepines, such as clonazepam 0.5-2 mg per day, may be used if antidepressants are ineffective
- B. Social phobia with performance anxiety (for specific situations known to be anxiety provoking) responds well to beta-blockers, such as propranolol. The effective dosage can be very low, such as 10- 20 mg qid. Or 20-40 mg given 30-60 minutes prior to the anxiety provoking event
- C. Cognitive/behavioral therapies are effective and should focus on cognitive retraining, desensitization, and relaxation techniques Combined pharmacotherapy and cognitive or behavioral therapies is the most effective treatment
- D. Group psychotherapy (Learn how to interact with other people)